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Phone

Fax

Specimen:

Received:

Spec Type: SURGICAL P

Status:

Subm Dr: -

Req#:

D.

RIGHT BREAST CANCER - INVASIVE

UID: 02B4C31E-38F8-4AF3-8DAD-527D5F27C11A
TCGA-A2-A1G6-01A-PR

Redacted

DATE:

DOCTOR(S):

PROCEDURE: MASTECTOMY MODIFIED RADICAL

- A. RIGHT MODIFIED RADICAL MASTECTOMY
- B. MEDIAL SKIN RIGHT BREAST

PART A IS RECEIVED LABELED [REDACTED] RIGHT MODIFIED RADICAL MASTECTOMY STITCH MARKS 12 O'CLOCK, IS A RIGHT MODIFIED RADICAL MASTECTOMY SPECIMEN MEASURING 26 X 18.5 X 5.5 CM IN GREATEST DIMENSIONS. THE NIPPLE IS UNREMARKABLE AND IS WITHIN A 19 X 11.7 CM SKIN ELLIPSE. IN THE 5 TO 6 O'CLOCK AREA AT THE INFERIOR MARGIN THERE IS A 3 CM IN DIAMETER CYSTIC STRUCTURE. THE SKIN HAS A BROWN DISCOLORED PLAQUE-LIKE AREA AT 12 O'CLOCK. THERE IS A PALPABLE FIBROUS MASS IN THE BREAST TISSUE BENEATH THE SUTURE SPANNING A DISTANCE OF 6 X 5 X 3.5 CM. THE SUPERFICIAL ASPECT IS MARKED WITH BLUE INK, THE DEEP MARGIN WITH BLACK. THE AXILLARY TAIL IS REMOVED FROM THE SPECIMEN AND WHERE IT IS REMOVED IT IS MARKED WITH RED INK AND DOES NOT REPRESENT TRUE MARGIN. THERE IS ONE GROSSLY POSITIVE LYMPH NODE IN THE MIDPORTION OF THE AXILLARY TAIL. THE HIGHEST NODE GROSSLY IS NEGATIVE. SECTIONS ARE SUBMITTED AS FOLLOWS:
A1--NIPPLE, A2--ONE-HALF OF A GROSSLY POSITIVE LYMPH NODE (REMAINDER SUBMITTED PER PROTOCOL), A3--GROSSLY HIGHEST LYMPH NODE, A4--ONE LYMPH NODE BISECTED, A5--ONE LYMPH NODE BISECTED, A6--ONE LYMPH NODE. A7--TUMOR AND DEEP MARGIN, A8 AND A9--TUMOR AND SKIN ADDING TO A8 AN AREA OF COIL CLIP, A10--TUMOR AT INFERIOR ASPECT ADJACENT TO FIBROUS TISSUE, A11--TISSUE IMMEDIATELY ADJACENT TO TUMOR UPPER INNER QUADRANT, A12--TISSUE IMMEDIATELY ADJACENT TO TUMOR UPPER OUTER QUADRANT, A13--ADDITIONAL UPPER OUTER QUADRANT, A14--LOWER OUTER QUADRANT 4.5 CM FROM GROSS TUMOR, A15--LOWER INNER QUADRANT 3 CM FROM GROSS TUMOR, A16--CYSTIC STRUCTURE IN 5 O'CLOCK AREA. THE AXILLARY TAIL IS THEN FURTHER EXAMINED FOR LYMPH NODES, WHICH ARE SUBMITTED AS FOLLOWS:
A17--ONE NODE BISECTED, A18--ONE BONE BISECTED, A19--ONE NODE BISECTED, A20--ONE NODE BISECTED. ASSOCIATED WITH THE PLAQUE-LIKE AREA OF THE SKIN IS AN UNDERLYING FIRM PINK-GRAY MASS MEASURING 3.5 X 3.3 X 3.1 CM IN GREATEST DIMENSIONS. GROSSLY THIS EXTENDS TO WITHIN 0.8 CM OF THE DEEP MARGIN. THE CENTRAL PORTION OF THE BREAST ALSO DEMONSTRATES DENSE GRAY-TAN FIBROUS TISSUE WITH MULTIPLE CYSTS.

ADDITIONAL SECTIONS ARE SUBMITTED AS FOLLOWS FOLLOWING INITIAL REVIEW OF SLIDES: A21 AND A22--SKIN MARGIN, A23--SUPERFICIAL MARGIN.

Chief of Pathology

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Patient: [REDACTED]

(Continued)

Specimen:

Received:

Spec Type: SURGICAL P
(Continued)

Status:

Subm Dr:

Req# [REDACTED]

PART B IS RECEIVED [REDACTED] MEDIAL SKIN RIGHT BREAST. IT CONSISTS OF AN UNORIENTED, SOMEWHAT TRIANGULAR PORTION OF SKIN WITH UNDERLYING SUBCUTANEOUS TISSUE, APPROXIMATELY 2 X 1 X 1 CM. THE RESECTION IS INKED. THE SPECIMEN IS SECTIONED AND ENTIRELY SUBMITTED IN CASSETTES B1 AND B2.

PROCEDURES:

88305, 88309, A BLK/23, B BLK/2

PART A RIGHT MODIFIED RADICAL MASTECTOMY: IN SITU AND INFILTRATING DUCTAL CARCINOMA. THE INFILTRATING TUMOR IS MODERATELY DIFFERENTIATED, NUCLEAR GRADE III/III WITH A MODERATE MITOTIC INDEX. THE SUPERFICIAL ASPECT OF THE TUMOR EXTENDS INTO THE DERMIS. THE DEEP MARGIN IS FREE OF TUMOR BY A DISTANCE OF 10 MM FOR A SUPERFICIAL TO DEEP SIZE OF 35 MM. OVER 90% OF THE TUMOR IS INVASIVE. AREAS OF PROBABLE BREAST LYMPHATIC SPACE INVASION ARE PRESENT BUT DERMAL LYMPHATIC SPACE INVASION IS NOT IDENTIFIED. BREAST TISSUE AWAY FROM THE PRIMARY TUMOR MASS SHOWS PROLIFERATIVE FIBROCYSTIC DISEASE. SKIN MARGINS CLOSEST TO THE TUMOR ARE NEGATIVE. METASTATIC CARCINOMA IS PRESENT IN 4 OF 9 LYMPH NODES.

PART B MEDIAL SKIN OF RIGHT BREAST, RESECTION: SKIN AND SUBCUTANEOUS FAT WITH NO EVIDENCE OF MALIGNANCY. (C)

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Signed _____

(signature on file) _____

(prelim.)

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
IIIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	lll	lll
Date Reviewed	12/1	